

Avir at Overton

1110 Hwy 135 S, Overton, TX 75684 · CCN 675408 · 100 beds · Organization ownership · Avir Health Group

CMS OVERALL

2 of 5

IMMEDIATE JEOPARDY

13 citations

SPECIAL FOCUS

SFF · CMS flagged

ABUSE ICON

Active

FINES (REPORTED)

\$340,621

BOTTOM LINE

Public CMS records show a high-risk pattern: Special Focus Facility status, an active abuse icon, 13 Immediate Jeopardy citations, and more than \$340,621 in fines — with RN staffing still thin, 42% of days showing zero RN hours, and a large gap between self-reported and payroll-based hours. Visit carefully and press for current corrections before deciding.

Comparatively less alarming

- Quality-measure star rating is 5 of 5 — better than inspection and staffing stars on selected clinical measures.
- Staffing stars (3 of 5) are higher than health-inspection stars (1 of 5) — still verify weekend and night RN coverage on tour.
- Several long-stay measures show low reported rates for catheters, UTIs, restraints, and/or depressive symptoms (0% in this extract).

Questions & concerns

- Immediate Jeopardy + abuse: Repeated F-0600 abuse citations across 2023–2025, including complaint surveys.
- Special Focus Facility: CMS has flagged this home for persistent quality problems — ask what the current SFF plan requires.
- Staffing & ownership: RN hours ~0.28/day; 42% of days with zero RN; self-report vs PBJ gap ~52%. Ownership changed 01/01/2024 (Winnie-Stowell Hospital District). Murrell portfolio averages high fines.

What to do next

1) Read the inspection story and F-tag table (pages 5–6). 2) Tour with the visit checklist and ask about weekend RN coverage, abuse policies, and SFF status. 3) Fill the visit checklist (page 8), then compare nearby options and use the decision worksheet (page 9) before signing anything.

Inside this brief: Scorecard · Staffing & ownership · Care fit · Inspection story · F-tag table · Penalties & safety · Visit checklist · Decision worksheet & sources

SCORECARD · HOW TO READ THESE RATINGS

CMS data as of August 2026 · Report prepared September 15, 2026 · Oversight composite uses public CMS inputs (higher = more concern in this model)

2 of 5

OVERALL CMS

1 of 5

HEALTH INSPECTION

3 of 5

STAFFING

5 of 5

QUALITY MEASURES

OVERSIGHT COMPOSITE

87.6 · elevated concern

DEFICIENCIES ON RECORD

41 health

COMPLAINT INVESTIGATIONS

24

What each signal means (plain English)

- Overall (2 of 5): CMS combines inspection, staffing, and quality-measure results. The overall star is a blend — not a prediction for any one resident.
- Health inspection (1 of 5): Driven by standard surveys plus recent complaint and infection-control findings. 13 Immediate Jeopardy citations weigh heavily here.
- Staffing (3 of 5): Based on payroll-based journal (PBJ) hours per resident day and related measures. Thin RN coverage and many zero-RN days pull this down.
- Quality measures (5 of 5): Selected clinical indicators (long-stay and/or short-stay). Stronger or weaker than other domains — still not a full picture of daily care.
- Oversight composite (87.6): Our summary of public risk signals (deficiencies, penalties, ownership, staffing, quality). 87.6 = elevated concern. Use it as a conversation starter, not a verdict.

Look closer

- Inspection stars (1) and quality-measure stars (5) diverge — dig into why, don't average them away.
- Special Focus Facility and/or the abuse icon are separate CMS alerts beyond the star rating.
- Fines totaling \$340,621 show federal enforcement, not just paperwork findings.

What this does not tell you

- Whether a specific nurse or shift will be good for your family member.
- Whether corrections after an Immediate Jeopardy citation are still working today.
- Private-pay pricing, room availability, or therapy schedules.
- Everything that happens between surveys — public data lags real life.

Stars are signals, not guarantees. CMS updates Care Compare on a schedule; always confirm the "last updated" date on [medicare.gov](https://www.medicare.gov) when you verify.

STAFFING & OWNERSHIP

Staffing figures are hours of nursing care per resident per day (HPRD) from CMS sources. Higher hours generally mean more care time available — averages still hide thin nights and weekends.

3.77

TOTAL NURSE HPRD

0.28

RN HPRD

0.89

LPN HPRD

2.60

CNA / AIDE HPRD

Weekend RN

0.28

HPRD on weekends — weekend total
nurse HPRD: 3.62

Days with zero RN

42.4%

Share of days with no registered nurse hours in recent PBJ data.

Self-report vs PBJ

52% gap

Self-reports ~ 0.26 RN HPRD; PBJ is lower when a gap is flagged. Ask which number they stand behind.

More staffing context

- Case-mix adjusted total HPRD: 4.44 · adjusted RN: 0.33
- Total nurse turnover: 58.3% (admin turnover reported as 1)
- Recent trend (PBQ quarters): RN hours moved 0.07 !' 0.17 !' 0.22 !' 0.13 ; zero - RN (the latest quarter shown).
- 2026-04 survey cited F-0727: registered nurse not on duty as required (scope/severity E) — corrected 2026-04-30 on paper.

Ownership

- Type: Organization
- Chain: Avir Health Group
- Owners on record: 10
- Change: 01/01/2024 !' Winnie-Stowell H District
- Worst linked owner: Murrell, Edward — controls 77 facilities; portfolio avg fines \$95,254; avg stars 2.64; 46.6% of that portfolio below average stars
- Related-party costs: \$33,696 (2024)
- No PE / REIT flags in this extract

Why it matters: Thin RN coverage and many zero-RN days affect clinical oversight nights and weekends. Ownership changes and large multi-facility portfolios can shift priorities — ask who is accountable on-site today and how any improvement plan is funded.

CARE FIT · WHO THIS PROFILE SPEAKS TO

Use this page to match the home’s published profile to your family member’s needs. Short-stay (rehab) and long-stay (custodial) experiences can differ — this extract has long-stay quality measures primarily.

100

CERTIFIED BEDS

49.4

AVG DAILY CENSUS

5 of 5

QM DOMAIN STARS

Long-stay relevance

Most published quality measures here are long-stay. If your loved one may remain months or longer, these indicators matter more than a short rehab stay. Still verify therapies, physician visits, and

Short-stay note

No short-stay quality-measure values appear in this extract (beyond vaccine rates, if any). If the stay is for post-hospital rehab, ask the admissions team for their short-stay outcomes and therapy staffing

activities on tour directly — don't rely on long-stay QM stars alone. Lower is generally better unless noted. These are facility-reported rates CMS publishes — not a complete clinical chart.

Measure	Facility rate	Reading tip
Falls with major injury (long-stay)	4.55%	Ask how call lights and fall huddles work on nights/weekends
Pressure ulcers (long-stay)	6.26%	Ask about turning schedules and wound-care nurse coverage
Need for help with daily activities increased	5.88%	Tracks functional decline — ask how care plans are updated
Ability to walk independently worsened	4.55%	Relevant if mobility is a goal
Significant weight loss (long-stay)	6.06%	Ask how dining assistance is staffed
New or worsened incontinence (long-stay)	11.98%	Ask about toileting schedules and response time

Also reported (context)

- Weight loss: 6.06% · New/worsened incontinence: 11.98% · Antianxiety / hypnotic meds: 32.35%
- Catheter left in, UTI, Restraints, Depressive symptoms: 0% in this extract

Fit checklist for your visit

- Is the need short rehab or long-term residence?
- Are memory-care / behavioral needs present? (abuse & antipsychotic flags matter more)
- Can family visit evenings and weekends when RN hours are thinner?
- Does census (well below capacity) feel calm or under-staffed on the floor?

INSPECTION STORY

Inspectors and complaint investigators have returned repeatedly. The pattern below is the story behind the 1-star inspection rating — read it before the F-tag table on the next page.

9 Abuse, Neglect, and Exploitation	8 Quality of Life and Care	4 Resident Rights	13 Immediate Jeopardy total
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PATTERN · IMMEDIATE JEOPARDY / ABUSE

Finding: Multiple F-0600 “protect from abuse” citations in 2023 and 2025.

Why it matters: Immediate Jeopardy means inspectors judged residents at risk of serious harm. An active CMS abuse icon signals ongoing concern, not a closed chapter.

Status: Several items show correction dates in the extract; confirm live status on Care Compare and during your visit. Latest row: Corrected.

Ask: "Walk me through the current abuse-prevention policy, who investigates allegations, and what changed after each IJ."

MAY 2026 · F-0607 · SCOPE J (IJ)

Finding: Develop and implement policies and procedures to prevent abuse, neglect, and theft.

Why it matters: Written policies only protect residents if staff know them and they are used after every allegation.

Status: Corrected 2026-05-27 per CMS extract.

Ask: "Walk me through the current abuse-prevention policy, who investigates allegations, and what changed after each IJ."

AUG 2025 · F-0689 · SCOPE J (IJ)

Finding: Ensure that a nursing home area is free from accident hazards and provides adequate supervision to prevent accidents.

Why it matters: Supervision gaps show up as falls, wandering, and equipment hazards — especially on thin RN shifts.

Status: Corrected per CMS extract.

Ask: "How are fall and hazard huddles run on evenings and weekends, and who is the RN on duty today?"

Also on record: 2023-10 Immediate Jeopardy for late family/physician notification (F-0580, corrected 2023-10-27) — ask for today’s written after-hours call standard. Category counts above are top CMS deficiency categories; a count is not the same as severity. Full ledger continues on page 6.

F-TAG TABLE · DECISION-RELEVANT DEFICIENCIES

Most serious and recent health deficiencies from the public extract (prioritizing Immediate Jeopardy). Plain labels paraphrase official CMS text; F-tag codes stay official. Full CMS record may contain more rows.

Date	F-tag	Plain label	Scope / severity	Status
2026-05-20	F-0607	Abuse-prevention policies & procedures	J · IJ · Isolated	Corrected 2026-05-27
2025-08-13	F-0600	Protect residents from abuse / neglect	J · IJ · Isolated	Corrected
2025-08-13	F-0689	Accident hazards / adequate supervision	J · IJ · Isolated	Corrected
2025-01-26	F-0600	Protect residents from abuse / neglect	J · IJ · Isolated	Corrected 2025-02-06
2025-01-26	F-0678	Basic life support / CPR before EMS	J · IJ · Isolated	Corrected 2025-02-06
2023-10-26	F-0600	Protect residents from abuse / neglect	K · IJ · Pattern	Corrected 2023-10-27
2023-10-26	F-0607	Abuse-prevention policies & procedures	K · IJ · Pattern	Corrected 2023-10-27
2023-10-26	F-0867	Set up an ongoing quality assessment and assurance group to review qu...	K · IJ · Pattern	Corrected 2023-10-27
2023-10-26	F-0580	Immediate family / physician notification	J · IJ · Isolated	Corrected 2023-10-27
2023-10-26	F-0689	Accident hazards / adequate supervision	J · IJ · Isolated	Corrected 2023-10-27
2023-10-26	F-0695	Safe respiratory care	J · IJ · Isolated	Corrected 2023-10-27
2023-08-16	F-0600	Protect residents from abuse / neglect	K · IJ · Pattern	Corrected 2023-09-07

* “Not corrected” reflects the correction flag in the public extract as of the report build — confirm live status on Care Compare and with the facility. IJ = Immediate Jeopardy. This table is a curated subset of 41 health deficiencies on record; complaint-driven surveys are common in this file.

PENALTIES & SAFETY EXTRAS

Federal money penalties and payment denials are separate from star ratings. They show CMS used enforcement tools — amounts and dates below come from the public penalty timeline.

TOTAL FINES
\$340,621 · 5 fines

PAYMENT DENIALS
None in extract

CMS ALERTS
SFF + Abuse icon

Penalty timeline

Date	Type	Detail
2026-05-20	Fine	\$19,615
2025-08-13	Fine	\$8,431
2025-08-13	Fine	\$14,325
2025-01-14	Fine	\$130,686
2023-10-26	Fine	\$167,564

Special Focus Facility (SFF)

CMS has placed this home on the Special Focus Facility list for persistent quality problems. SFF homes receive more frequent oversight. Ask for the current SFF action plan, recent revisit results, and whether CMS has moved them toward graduation or deeper enforcement

Abuse icon — active

Care Compare displays an abuse icon when certain abuse-related criteria are met. Combined with abuse-related citations, treat this as a must-ask topic on every tour — policies, training, reporting timelines, and family communication.

Fire safety (when present)

- 12 fire-safety deficiencies on record · fire safety score 59 · 1 fire Immediate Jeopardy count in extract
- Example (2025-02-18, tag 0521, scope C): Ensure heating and ventilation systems that have been properly installed according to the manufacturer's instructions. — not marked corrected in extract
- Ask facilities staff to show recent fire-drill logs and any open Life Safety Code plans of correction

Fine totals on marketing pages sometimes round differently than the detailed timeline. This brief uses the facility total_fines field from the CMS extract.

VISIT CHECKLIST · QUESTIONS TIED TO THIS FACILITY

Bring this page on the tour. Every question links to a real flag in the CMS record. Write who answered you. Nearby facilities to compare are on page 9.

1. You are a Special Focus Facility with an active abuse icon — what does the current SFF plan require, and what has improved since the last revisit?

Answered by: _____ Role / date: _____

2. F-0600 abuse citations recur from 2023 into 2025 — who investigates allegations today, and may I see the written prevention policy?

Answered by: _____ Role / date: _____

3. About 42% of days show zero RN hours — who is the registered nurse covering this shift and this coming weekend?

Answered by: _____ Role / date: _____

4. Self-reported RN hours differ from PBJ by roughly 52% — which figure do you stand behind, and why the gap?

Answered by: _____ Role / date: _____

5. After the F-0689 accident-hazard Immediate Jeopardy, how do you review call lights and falls on evenings and weekends?

Answered by: _____ Role / date: _____

6. Family notification was cited at Immediate Jeopardy (F-0580) — what is your after-hours standard if my relative declines?

Answered by: _____ Role / date: _____

7. Ownership changed to Winnie-Stowell Hospital District on 01/01/2024 — who is accountable on-site day to day, and how does Avir Health Group relate?

Answered by: _____ Role / date: _____

8. When a CNA or RN calls out, do you limit new admissions or use agency staff — and may I speak with a family-council contact?

Answered by: _____ Role / date: _____

Tip: photograph any written policy they hand you, and note the staff member's name on each line above before you leave.

DECISION WORKSHEET & SOURCES

After the tour, capture what must be true for your family — then verify on Care Compare.

Nearby alternatives to compare

Facility	City	Stars	Composite	Fines	IJ
Arbor Grace Guest Care Center	Kilgore	1	41.4	\$187,096	1
Willow Rehab & Nursing	Kilgore	3	78.7	\$219,229	3
Henderson Health & Rehabilitation Center	Henderson	2	68.8	\$10,546	3

Not recommendations — starting points for Care Compare. Lower composite = less aggregated concern in this model.

Must-haves

Non-negotiables (weekend RN, abuse-reporting path, SFF progress)

Tradeoffs we can accept

Imperfect but workable for your situation

People to call

Admissions · DON · social work · ombudsman

Compare & follow-up

Other facilities / dates to re-check

Sources

CMS Care Compare / Provider Data Catalog (stars, ownership, SFF, abuse icon) · CMS PBJ (nursing hours, weekend RN, zero-RN days) · CMS deficiencies & penalties (F-tags, fines, denials, fire safety). Avir at Overton · CCN 675408 · CMS as of August 2026 · Report September 15, 2026.

Limitation: This report summarizes public data and cannot predict an individual resident’s experience. Confirm current conditions during a visit. · The Oversight Report · oversightreports.com · DataLink Clinical LLC